

1. Administrative & Study Identification

Full Study Title: A Randomized, Phase 3, Open-label Study to Evaluate Sigvotatug Vedotin Compared With Docetaxel in Adult Participants With Previously Treated Non-small Cell Lung Cancer (Be6A Lung-01) **Short Title/Acronym:** A Study of SGN-B6A Versus Docetaxel in Previously Treated Non-small Cell Lung Cancer **Protocol Number:** IRAS ID 1008246 **NCT Number:** NCT06012435 **Posting ID:** 30912442171855774 **Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING) **Sponsor Name:** Pfizer **Investigational Product:** Sigvotatug vedotin (SGN-B6A, an antibody-drug conjugate) / Docetaxel **Phase of Development:** Phase 3 **Medical Monitor:** Scott Knowles (EU-Regulatory@seagen.com) / Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy, primarily Overall Survival (OS) and Progression Free Survival (PFS), of sigvotatug vedotin compared with docetaxel in adult participants with previously treated nonsquamous non-small cell lung cancer (NSCLC). **Secondary Objectives:** To assess the Confirmed Objective Response Rate (ORR), Progression-Free Survival (PFS) by investigator assessment, Time to Deterioration (TTD) in Quality of Life scores (EORTC QLQ-LC13, QLQ-C30), and the overall safety and tolerability of the therapies.

2.2 Background & Rationale To address the need for more efficacious treatments in patients with heavily pretreated NSCLC. Sigvotatug vedotin is an antibody-drug conjugate (ADC) targeting integrin beta-6 with a monomethyl auristatin E payload, offering a broader therapeutic index and targeted delivery. Prior Phase 1 data (SGNB6A-001) showed promising activity with a 31% ORR in heavily pretreated, taxane-naïve NSCLC patients, warranting a Phase 3 comparison against standard-of-care docetaxel.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Docetaxel) **Blinding:** Open-label **Randomization:** 1:1 Randomized (50% chance for either arm)

3.2 Planned Enrollment & Duration Target \$N\$: 703 participants **Number of Sites:** ~210 locations across 28 countries **Estimated Study Start:** 21-Feb-2024 **Estimated Primary Completion:** 12-Mar-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Histologically or cytologically confirmed diagnosis of locally advanced, unresectable (Stage IIIB, IIIC), or metastatic Stage IV (M1a, M1b, or M1c) NSCLC with nonsquamous histology. 3. Previously treated with no more than a platinum-based chemotherapy and an anti-PD-(L)1 drug. (Participants with treatable genomic alterations must have had at least 1 drug for that alteration).

4.2 Exclusion Criteria 1. Life expectancy of less than ($<$) 3 months. 2. Prior treatment with antimicrotubule agents (taxanes, vinca alkaloids, or MMAEs) in the locally advanced, unresectable/refractory, or metastatic setting. 3. Active central nervous system (CNS) lesions, including leptomeningeal metastasis. 4. Any systemic anticancer therapy (standard or experimental) within 21 days prior to Cycle 1 Day 1, or prior radiation therapy to the lung parenchyma >30 Gray (Gy) within 6 months.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * *Experimental Arm:* Sigvotatug vedotin administered on Days 1 and 15 during each 28-day cycle. * *Active Comparator Arm:* Docetaxel (ATC Code: L01CD02 | Trade Names: Docefrez, Taxotere | EMA URLs: [Docefrez](#), [Taxotere](#)) administered on Day 1 of each 21-day cycle. **Route of Administration:** Intravenous infusion. **Duration of Treatment:** Until disease progression, unacceptable toxicity, or completion of the approximate 5-year study timeframe.

5.2 Concomitant & Prohibited Therapy Permitted: Stable dose of ≤ 10 mg/day of prednisone or equivalent (if requiring steroid treatment) for at least 2 weeks prior. **Prohibited:** Systemic anticancer therapy within 21 days prior to Cycle 1 Day 1.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -21 to 0	Informed Consent, Medical History, Physical Exams, Baseline Scans (MRI/CT)
Baseline	Day 0	Randomization to Sigvotatug Vedotin or Docetaxel, First Dose
Interim	Cycle Days	Physical exams, EORTC QLQ-LC13/C30 questionnaires, urine/blood samples, ECGs, and imaging scans (MRI/CT/Bone scans)
End of Study	Up to 5 Years	Long-term follow-up for Overall Survival, other NSCLC therapies, and participant status

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints

- 1. Overall Survival (OS):** Evaluated between the experimental arm (sigvotatug vedotin) and control arm (docetaxel) in all participants and in participants whose tumors express high levels of integrin beta-6 (IB6-high). Measurement Method: Time from the date of randomization to date of death due to any cause.
- 2. Progression Free Survival (PFS):** Assessed per RECIST v1.1 by Blinded Independent Central Review (BICR) between the experimental and control arms in all participants and in the IB6-high subgroup. Measurement Method: Time from date of randomization to the first documented disease progression per RECIST v1.1 or to death due to any cause.

7.2 Secondary & Exploratory Endpoints

- * Confirmed Objective Response Rate (ORR) per RECIST v1.1 as assessed by BICR in all participants and in the IB6-high subgroup.
- * Confirmed ORR per RECIST v1.1 by investigator assessment.
- * PFS per RECIST v1.1 by investigator assessment.
- * Time to Deterioration (TTD) in the dyspnea, cough, and chest pain scores on the EORTC QLQ-LC13, and global health status/QoL on the EORTC QLQ-C30.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of treatment-emergent adverse events through regular physical exams, electrocardiograms, and laboratory sampling (blood and urine).

Serious Adverse Events (SAEs): Standard protocol-defined reporting timelines.

Data Monitoring Committee (DMC): Overseen by the Sponsor to monitor the safety and efficacy endpoints continuously.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all primary efficacy outcomes. Safety analysis set for adverse events tracking.

Sample Size Justification: $N = 703$ participants to adequately power the study for the primary efficacy (Overall Survival and PFS) comparisons.

Handling of Missing Data: Appropriate time-to-event statistical methodologies (e.g., Kaplan-Meier estimation with right-censoring) for OS and PFS endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Scheduled onsite and centralized monitoring visits across the ~210 global locations to ensure protocol

adherence. **Audit Trail:** eCRF documentation, central imaging reviews (BICR), and data clarification systems.

11. Study Identifiers & Governance

Primary ID: 1008246 **Registry Number:** NCT06012435 **Posting ID:** 30912442171855774 **Sponsor/Lead Organization:** Pfizer **Study Title:** Be6A Lung-01 **Official Regulatory Title:** A Randomized, Phase 3, Open-label Study to Evaluate Sigvotatug Vedotin Compared With Docetaxel in Adult Participants With Previously Treated Non-small Cell Lung Cancer

12. Clinical Framework (NSCLC Specific)

Primary Condition: Carcinoma, Non-Small-Cell Lung **Preferred UMLS Name:** Drug Therapy for Non-small Cell Lung Carcinoma **Condition Semantic Details:** Carcinoma (Semantic Type: Neoplastic Process). **Condition Definition:** A malignant tumor arising from epithelial cells. Carcinomas that arise from glandular epithelium are called adenocarcinomas, those that arise from squamous epithelium are called squamous cell carcinomas, and those that arise from transitional epithelium are called transitional cell carcinomas (NCI Thesaurus). **Disease Codes:** MeSH: D002277 | HPO: HP:0030731 | SNOMED ID: 1187425009 **Diagnosis Threshold:** Histologically or cytologically confirmed locally advanced, unresectable (Stage IIIB, IIIC), or metastatic Stage IV (M1a, M1b, or M1c) disease. **Medical Methodology:** Antibody-Drug Conjugate (ADC) vs. standard cytotoxic chemotherapy **Optimization Target:** Targeting Integrin beta-6 for targeted MMAE payload delivery

13. Study Procedures & Methodology

Management Pathway: Second-line or later-line treatment pathway for advanced NSCLC **Education Protocol (HCP):** Management of anticipated ADC-related and docetaxel-related toxicities **Education Protocol (Patient):** Informed consent, schedule adherence (21-day vs. 28-day cycles), and QoL PRO questionnaires **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for tumor response scans

14. Observation & Follow-Up Schedule

Total Duration: Approximately 5 years **Onsite Visit Cadence:** Depending on the assigned arm (Days 1 and 15 of 28-day cycle, or Day 1 of 21-day cycle) **Remote Monitoring Frequency:** Standard protocol safety follow-up for survival **Checklist Review Interval:** Regular intervals aligned with EORTC QLQ-LC13 and EORTC QLQ-C30 assessments

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					